

PSYCHOSOCIAL HAZARD MAPPING — SWITCHBOARD SERVICES

Work Health and Safety Act 2011 (Qld) · WHS Regulation 2011 (Qld) · Managing the risk of psychosocial hazards at work Code of Practice 2022

Cory Lea Shepherd — AO3, Switchboard Services, Logan Hospital · August 2026 · Prepared for the clause 7.2 risk assessment and consultation under ss 47–49. Companion to "Return to Work and Workplace Controls — Proposals for Consultation" (P-references below). Every hazard is described as a feature of the work and its systems; nothing here is an allegation against any person.

PART 1 — FRAMEWORK

Section 19 of the Act places the primary duty on the person conducting the business to ensure, so far as is reasonably practicable, the health of workers — health includes psychological health. The Regulation requires psychosocial hazards to be identified, risks assessed, controls implemented in the order of the hierarchy, and controls reviewed. Sections 47–49 require consultation with affected workers at each step. The Code supplies the recognised categories and data sources used below.

PART 2 — THE MAPPING

#	HAZARD (Code of Practice 2022)	HOW IT PRESENTS AT THE SWITCHBOARD	RISK	PROPOSED CONTROL (Proposals ref)
1	High job demands	200–300 calls per shift, many in distress or urgency; multiple concurrent systems; emergency codes through the shift; functions absorbed when other support is unavailable	Cognitive overload; sustained hyper-vigilance; exhaustion	Evidence-based rostering matched to demand peaks (P7); escalation and backup pathways so absorbed functions return to their owners (P1, P4)
2	Low job control	Operational changes take effect without consultation with the operators who work the board; no operator input into roster design	The high-demand / low-control combination the Code identifies as high risk	Consultation before operational change (P2); real-time team consultation within supervisor decisions (P6); operator input to roster design (P7)
3	Poor support	Supervision and decision-support not reliably available in real time, especially after hours and weekends, in continuous work	Unsupported decision-making in safety-critical moments; accumulated strain	Integrated manager in hours (P3); 24/7 on-shift supervisor — authority follows the clock (P6); structured handover so support context transfers (P8)
4	Low role clarity	Functions accrete to the Switchboard without documented boundaries (complaints, departmental support, roster administration)	Role overload; blame absorbed for matters outside the role	Documented boundaries and escalation paths (P1, P4); decision log and handover making responsibilities explicit (P8)
5	Poor organisational change management	Process and system-access changes made without consulting affected operators; effects discovered in live operation	Preventable failures experienced at the console; erosion of trust	Consultation before operational change, systematised (P2)
6	Inadequate reward and recognition	No development pathway within the room; competence and added responsibility carry no recognition	Effort–reward imbalance; disengagement; loss of experienced staff	Rotating on-shift supervisor function with in-charge recognition and a trained pathway (P6)
7	Poor organisational justice	Inconsistent process standards; complaints and workplace matters raised without acknowledgment, outcome or feedback	The experience of unfairness — a recognised hazard in its own right	Complaint-response standard (P4); one rulebook for the room (P5); written feedback on matters raised
8	Remote / isolated work (after hours)	After-hours and weekend shifts without present supervision or management support, functions unchanged	Unsupported exposure to all other hazards in the periods of least support	24/7 on-shift supervisor function (P6); handover continuity across shifts (P8)
9	Distressed and aggressive callers	First point of contact for the hospital; many contacts made in distress, fear, conflict or crisis; complaints often expressed as accusations to the operator	Cumulative exposure to occupational violence and aggression in verbal form	Exposure ends at triage: complaint standard and escalation pathway (P1, P4); exposure named in the assessment scope

#	HAZARD (Code of Practice 2020)	HOW IT PRESENTS AT THE SWITCHBOARD	RISK	PROPOSED CONTROL (Proposals ref)
10	Fatigue	Rostering capable of producing breaks below the mandated minimum; and — observable across the team — fatigue currently managed informally by staff themselves through personal leave taken on and after high-volume periods. The pattern is visible when the rosters are read against the leave records and the demand data: the cost of the unmanaged hazard is transferred to workers' own leave balances	Impaired performance in a safety-critical function; cumulative harm; depletion of leave entitlements as the de facto fatigue control	Fatigue rules built into roster design — minimum breaks enforced structurally, so recovery is provided by the roster, not purchased by the worker (P7)

PART 3 — CONSULTATION AND REVIEW

This mapping is offered as a worker's contribution to hazard identification under ss 47–49 — an input to the assessment, not a substitute for it. I ask that the assessment adopt the Code's categories; draw on the Code's data sources (complaints records, absence and turnover data, EAP usage, workload and call-volume data); specifically, map the team's leave usage against the rosters and the demand data, which will show whether fatigue is presently being absorbed through personal leave; be conducted by an assessor independent of the functions assessed; select controls in consultation in the order the Regulation requires; and provide implemented controls with review dates, with the affected operators included in the review.

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